Using coercion in mental disorders or risking the patient's death? An analysis of the protocols of a clinical ethics committee and a derived decision algorithm

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ABSTRACT

While principle-based ethics is well known and widely accepted in psychiatry, much less is known about how decisions are made in clinical practice, which case scenarios exist, and which challenges exist for decision-making. Protocols of the central ethics committee responsible for four psychiatric hospitals over 7 years (N=17) were analysed. While four cases concerned suicide risk in the case of intended hospital discharge, the vast majority (N=13) concerned questions of whether the responsible physician should or should not initiate the use of coercion in patients lacking mental capacity. The committee's recommendations were non-uniform. Forced feeding and electroconvulsive therapy were endorsed in each one case. In two cases of intermittent loss of capacity due to heavy drinking or intermittent severe suicidal ideation, a self-binding contract was recommended and the use of coercion was considered as justified for a very limited period. In all other cases, most of which involved involuntary treatment, the use of coercion was not endorsed. Without exception, the recommendations were accepted with relief by the physicians and their treatment teams, who feared liability in the event of harm to the patient. Eventually, a model of a decision algorithm was derived from the ethical arguments in the protocols.

INTRODUCTION

The use of coercion is an issue that has accompanied psychiatry throughout its history as an ethical challenge.[1] Coercion is not unique to psychiatry as a medical specialty, but can be considered necessary in some instances in all medical specialties.[2] However, only psychiatry (and to a lesser extent neurology) treats disorders that affect the organ that makes decisions, the brain. With the exception of some serious neurological disorders such as cerebral haemorrhage and encephalitis, all disorders that can affect mental capacity are mental disorders.[3] At first glance, this statement may seem exaggerated. Many serious somatic illnesses can cause subsequent disturbances in brain function, such as surgery in the elderly, infections or anti-immune diseases. However, the pathophysiological causes of disorientation or agitation are not hip surgery or pneumonia, but subsequent secondary delirium or encephalitis, which are psychiatric and neurological disorders, respectively. Approaches of medical ethics agree that any action taken against a patient's will can only be justified if the patient's capacity to make decisions (mental capacity) is impaired by a mental disorder. This view is increasingly reflected in the legislation of many developed countries, which define lack of capacity (rather than the presence of a mental disorder) as a necessary criterion for acting against a patient's will. How psychiatrists can objectively determine lack of capacity is an ongoing debate that will not be pursued here.[4–6] Nor will we go into the intense debate between psychiatrists, ethicists and lawyers on the one hand, and major international organisations such as the United Nations (UN) Committee on the UN Convention on the Rights of Persons with Disabilities (UN-CRPD) and the WHO, as well as some patient organisations, on the other, about whether there is such a thing as lack of capacity and whether it can justify the use of coercion.[7–9] According to widely accepted ethical considerations,[10] the use of coercive interventions can be not only accepted but even necessary, when the patient endangers himself or herself or other people, caused by a mental disorder that impairs the decision-making capacity (principle of autonomy to be taken into account). In such situations, it is necessary, to act in the patient's 'best interest' according to her or his presumable will (principles of beneficence and non maleficence to be taken into account), if there is no evidence of a withstanding will of the patient, for eample, in an advance directive or a joint crisis plan.[11 12] However, lack of capacity does not automatically justify medical acting in the 'patient's best interest' according to medical knowledge. Expected benefits must be weighed against expected adverse effects (taking into account the principle of non-maleficence). Furthermore, considerations of justice are necessary. For example, to treat a patient explicitly different from others due to gender, age, race, income or personal interests would raise ethical concerns. While these general principles are well known and broadly accepted in the field,[13] much less is known about how decisions are made in clinical practice, which case scenarios exist, and which challenges exist for decision making.[14] Although the general ethical principles are well known, a decision algorithm that takes into account concrete clinical aspects is not available so far. The author has been chairing a central clinical

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ethics committee for many years and takes the approach to analyse the available protocols from a 7-year-period.

METHODS

The clinical ethics committee

The ethics committee of the Centres for Psychiatry Suedwuerttemberg is responsible for four psychiatric hospitals with 10 sites and about 17 000 admissions per year. The hospital organisations encompass in-patient treatment, day-clinic treatment, outpatient treatment and outpatient living assistance, from low-frequent assisted living up to locked sheltered homes. According to the rules of procedure of the ethics committee, it includes four medical directors, three medical experts especially qualified for ethical issues, a representative of religious communities, a patient representative, a nurse with high ethical expertise, a non-medical external ethicist and a retired judge of a high court. According to the rules of procedure, any doctor working in the hospitals can call on the Ethics Committee at any time if he or she is faced with an ethical problem. In practice, a considerable amount of ethical expertise is available in all departments, and ethical advice from qualified experts is sought formally and informally at lower levels. The central ethics committee is therefore only consulted in very exceptional cases, usually after intensive discussion among the treatment teams and on the initiative of the responsible chief physicians. In most cases, the ethical conflicts have not only an ethical, but also a legal dimension, in that the physicians responsible are afraid of being held liable for their actions or omissions if this should result in harm to the patient.

When an application is sent to the ethics committee, it should be accompanied by a full case report and a clear ethical question. A meeting of the committee will be scheduled in good time and will be held as a videoconference. On the applicant's side, all interested team members can participate, as well as the patient's legal representative (if applicable) and the patient himself or herself, if he or she so wishes. After the applicant has presented the case and the ethical question, there will be a round of further questions from the committee members to the applicant. If appropriate, the applicant should explain to the non-medical members of the committee the available medical evidence relating to the proposed treatment. This is followed by the ethical discussion, structured according to principle-based ethics, but using also aspects of ethics of care,[15] until a consensus view is reached by the committee and a recommendation is made. If consensus cannot be reached, a minority opinion may be formulated. The chair of the committee will then draft a protocol, which will be sent to the applicant once consensus has been reached among the committee members on the protocol and recommendations. This should be completed within 1–2 weeks.

Materials

Between 2016 and 2023, 17 ethics committee meetings were held and the protocols are available. The protocols contain the detailed case reports, a section on ethical considerations with pros and cons according to the four principles of principle-based ethics (respect for patient autonomy, beneficence, non-maleficence, fairness) and an ethical recommendation with justification. If there is a minority opinion against the recommendation, this is mentioned along with the recommendations.

Research method

The protocols were analysed qualitatively using content analysis methods in relation to their main themes and recommendations. As the vast majority of ethical questions (13/17) focused on whether coercion should be used against the patient's will in the patient's best interest or not, these 13 protocols were used for further qualitative analysis. Using grounded theory[16] methods, both deductive and inductive, a theoretical decision algorithm was developed to identify those aspects that should be balanced to arrive at a reasonable ethical recommendation.

RESULTS

A synopsis of the cases is presented in table 1.

Diagnoses

We identified 17 cases with the respective protocols in a period of 7 years between September 2016 and September 2023. Table 1 presents a synopsis of the cases. The most frequent diagnosis was schizophrenia (eight times), followed by post-traumatic stress and personality disorders (five times), major depression, dementia, alcohol dependency, bipolar disorder and delusional disorder (each one). In two cases, the ethical questions referred to treatment of somatic diseases (metastatic lung cancer and severe chronic obstructive pulmonary disease).

Applicants

Thirteen out of 17 applications came from inpatient treatment, one from a forensic psychiatric service, one from a supported living service and one from a supported living service and an inpatient unit together. None came from outpatient treatment and day clinics. In reality, however, the separation was not so strict. Most of the patients presented had been known for many years in many treatment settings as inpatients, outpatients and also as users of community services.

Ethical questions

Four cases (Nos. 1–4) concerned the question of whether the patient could be discharged from hospital to a less supervised outpatient setting despite continuing suicidal acts or threats. These case scenarios were more or less similar and all involved patients with a personality disorder. Only these cases were not about coercion or, respectively, they referred to a form of coercion that not legally would be considered as coercion: The question was whether they could be discharged from hospital, against their expressed will to stay there. All other ethical questions related to whether or not one or more forms of coercion should be used. Three cases (Nos. 5–7) concerned treatment with electroconvulsive therapy against the patient's will, two cases (Nos. 8–9) concerned forced feeding, three cases (Nos. 10–12) concerned forced antipsychotic treatment, three cases (Nos. 11, 14, 16) concerned long-term involuntary admission to a sheltered facility, two cases (Nos. 13 and 17) concerned forced treatment of a somatic illness and one case (No. 15) concerned with repeated involuntary admission to hospital.

Risks

Danger to self was the issue of concern in all cases; only one case (No. 11) involved elements of danger to others. The feared risk was mostly death (Nos. 1–4, 7–10, 13–15, 17). Deterioration or lack of recovery from the mental disorder was the expressed risk in four cases (Nos 4,5,11,12), and in one case, it was severe neglect (No. 16).

Assessment of patient autonomy

Autonomy was assessed as decision-making capacity. In one case (No. 1), capacity was judged to be clearly present. In nine cases

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Table 1 Synopsis of cases

No	Diagnosis	Application from	Ethical question	EC recommendation
1	Personality disorder	Hospital	Can the patient be discharged from hospital in spite of suicide threats?	Yes, with capacity being considered as given
2	Complex post-traumatic stress disorder, borderline personality disorder	Hospital	Can the patient be discharged to an assisted living service, facing repeated suicide attempts?	Yes, with capacity mostly being considered as given, with sufficient preparation
3	Complex post-traumatic stress disorder, borderline personality disorder	Hospital	Can the patient be discharged to an assisted living service, facing repeated suicide attempts?	Yes, with capacity mostly being considered as given
4	Personality disorder	Hospital	Can the patient be discharged from hospital to assisted outpatient living inspite of repeated endangerment by inappropriate use of insulin?	Yes, with capacity being considered as mostly given except for short periods of crisis, short-term re-admissions should be offered
5	Schizophrenia	Hospital	ECT against the patient's will?	Yes, capacity not given
6	Schizophrenia	Hospital	ECT against the patient's will?	No, poor balance of pros and cons
7	Schizophrenia	Hospital and assisted living services	Continued ECT against the patient's will, facing continuing refusal of drinking and eating due to delusional reasons?	No, poor balance of pros and cons
8	Schizophrenia	Hospital	Feeding against the patient's will, facing repeated periods of refusal of nutrition with expressed intent to die?	No, capacity at least partially given and patient's wished to be respected
9	Major depression	Hospital	Feeding against the patient's will, facing refusal of food and drinking intake?	Yes, capacity probably not given, for 2 weeks, then re-evaluation
10	Schizophrenia, post-traumatic stress disorder	Hospital	Antipsychotic treatment against the patient's will, facing repeated suicide attempts?	Yes, with capacity not given, though reasonable biographical aspects being taken into account
11	Schizophrenia	Hospital	Prolongation of involuntary antipsychotic treatment and involuntary commitment in locked sheltered home, facing repeated unmotivated trips across Europe with endangerment of self and others?	Yes, with capacity not given. Minority opinion: Involuntary treatment, but no detention
12	Schizophrenia	Hospital	Involuntary antipsychotic treatment of a minor	No definite recommendation, committee split.
13	Schizophrenia lung cancer	Forensic hospital	Diagnostics, optionally treatment of cancer against the patient's will, facing delusional denial of illness?	No. Capacity not given, but poor balance of pros and cons
14	Post-traumatic stress disorder	Hospital	Involuntary commitment in sheltered psychiatric facility due to repeated severe suicide attempts required?	No, with capacity considered as mostly given except for short periods of crisis. Recommendation of a self-binding contract allowing involuntary commitment for max 2 days in these situations
15	Alcohol dependency	Assisted living service	Involuntary commitment required, facing repeated periods of heavy drinking with risk of fatality?	No, with capacity considered mostly given. Self-binding contract recommended to allow short-term involuntary commitment
16	Moderate dementia delusional disorder	Hospital	Involuntary commitment in sheltered home, facing failed attempts with outpatient nursing services?	No, not appropriate, no clear danger to self
17	Bipolar disorder, severe lung disease	Hospital	Transfer to somatic intensive care unit against the patient's will?	No, with capacity mostly not given. Poor balance of pros and cons

EC, ethics committee; ECT, electroconvulsive therapy.

(Nos. 5–13, 16), capacity was judged to be clearly lacking. The remaining cases (Nos. 2–4, 14, 15, 17), about one third, proved difficult in this respect. In cases 2–4 and 14, patients with a complex history of trauma and personality disorder, autonomy was generally considered to be present. However, there was clear clinical evidence that in periods of crisis they were so narrowed in their thinking that they could not control suicidal ideation, so that lack of capacity was assumed and paternalistic protective measures were deemed necessary. These periods were brief, varied in severity from patient to patient and, in the committee's view, never exceeded 2 days. Similarly, in case No. 15 of a patient with severe alcohol dependency, autonomy was considered to be generally given, but not in periods of heavy, life-threatening drinking, which caused ethical conflicts for the staff of the assisted living services. In other cases, autonomy had to be denied because of persistent, clearly delusional distortions of reality. However, a deeper understanding of the patients' histories revealed that their wishes (to die or to take risks) had reasonable aspects rooted in their personality and biography.

Recommendations

The ethical questions were denied (eight times) and endorsed (eight times) in equal proportions, one recommendation was ambiguous. In one case, a minority vote was expressed, and in one case of involuntary treatment of a minor, opinions within the committee remained controversial also after intensive discussion. Most recommendations included explanations and recommendations for further treatment, for example, preparations of discharge, alternative options of care to be considered etc. In two cases (Nos. 14, 15), the committee recommended that a self-binding advance directive (Ulysses contract)[17] should be agreed with the patient at the time of capacity. This should enable protective (and then coercive) measures to be taken in the patient's best interests during periods of lack of capacity and self-endangerment. If the patient would not agree to such an advance directive, the use of coercion would not be considered justified.

Decision-making model

The model that was developed based on all ethical arguments, according to principle-based ethics, is displayed in figure 1. The first element of the decision tree is decision-making capacity. When capacity is denied, ethical weighing begins with arguments visualised on a scale. In relation to beneficence and non-maleficence, medical evidence comes into play and has an important role. For example, in case 13, a patient with

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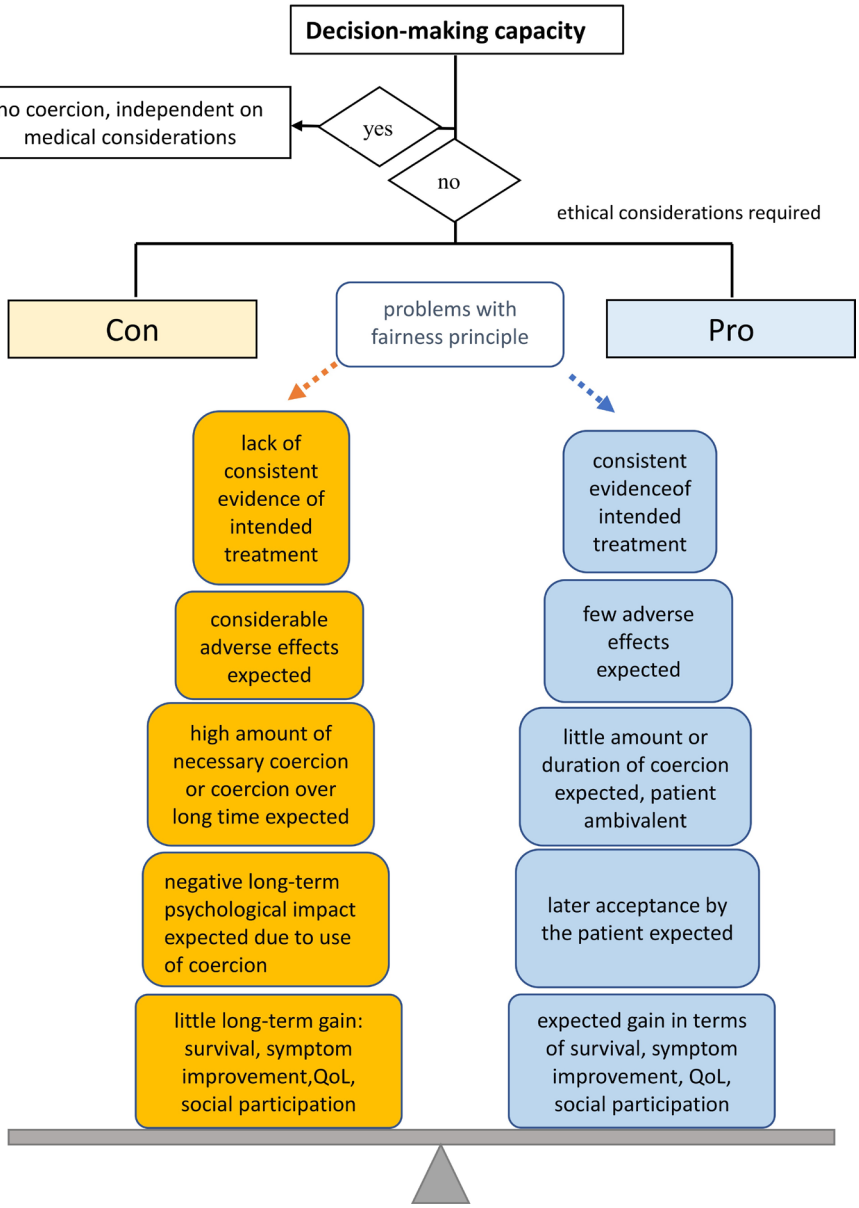

Figure 1 Decision algorithm for the use of coercion.

metastatic lung cancer, the use of coercion to determine further diagnosis and therapy was denied based on the knowledge of a very poor prognosis even with appropriate therapy. Aspects of patient autonomy are not all irrelevant, even when decision-making capacity has been denied. They return to ethical considerations in the form of the degree of coercion, both in terms of severity and duration, that a patient would be likely to suffer. If a patient is deeply ambivalent and experience shows that she will accept a decision against her expressed will as soon as a court says so, and, for example, then will take oral medication, the level of coercion can be considered comparably low. On the other hand, if the patient's refusal is very strong and, for example, it is likely that continuous restraint over weeks would be necessary to achieve forced feeding, the level of coercion must be considered very high and would need to be counterbalanced by very clear expected benefits.[18] In any case, the possible use of coercion should never be neglected in ethical considerations but weighs heavily on the side of maleficence. Consideration must be given not only to the short-term therapeutic gains made possible by the use of coercion, but also to the medium-term and long-term consequences of coercion on patients' self-esteem and attitudes towards treatment and psychiatric institutions. Finally, the model illustrates that in mental disorders, beneficence and maleficence cannot be determined solely from a medical perspective, but must follow a biopsychosocial model. In this perspective, the use of coercion is an important aspect of maleficence to be considered. Benefits, on the other hand, include not only medical aspects such as symptom improvement, but also quality of life, social integration and personal freedom. The principle of fairness was not ostensibly relevant in most cases, but may influence the decision one way or the other. For example, refraining from a coercive intervention while continuing hospital treatment may result in excessive use of staff resources over a long period of time at the expense of other patients. Staff exhaustion may also be an issue to consider. This sounds rather utilitarian, but rights of third parties need to be considered in ethical decisions also in cases that are not about danger to others. On the other hand, the use of coercion would seem questionable if it would be not considered necessary for other patients in a similar situation.

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DISCUSSION

The analysis of the minutes of an ethics committee produced some important results. First, the main ethical issue in psychiatry is whether or not to use some form of coercion on patients who lack capacity. This finding is based on an analysis of the cases that were considered the most difficult to decide over a period of 6 years, during which time over 100 000 cases were treated in the hospitals involved. Although most patients do not die from mental disorders other than dementia, except from suicide or starvation, exactly these were the risks discussed in all cases. Second, the analysis yielded a model that concretises principle-based ethics for the use of restraint in psychiatry and can be understood as a clinical decision support tool. This refers only to coercion on the basis of danger to self, as danger to others was not an issue here and may be easier to decide under the law. Third, the analysis shows that a good knowledge of the evidence is absolutely essential for appropriate ethical decisions in such cases. Evidence may be required in relation to the treatment of comorbid severe somatic disorders or the patient's mental disorder itself. Recognising that evidence is not infrequently inconclusive, particularly in chronic conditions,[19 20] evidence must be applied to the individual case, including the patient's perspective. Expected benefits and harms can only be identified and weighed against each other with such knowledge of the evidence. Fourth, ethical considerations about the use of coercion must necessarily be based on a broad understanding of mental disorders and reflect their biopsychosocial aspects, both cross-sectionally and longitudinally.[21] This includes not only somatic comorbidities and mental symptoms, but also subjective well-being, subjective and objective quality of life, social inclusion in the sense of the UN-CRPD, and personal freedom (not being institutionalised).[3] The cases analysed here strongly support the view that autonomy and capacity are not dichotomous concepts (which they are from a legal point of view), but often fluctuate over time, and that indicators of impaired capacity such as delusions can coexist with reasonable, personally rooted beliefs that should be taken seriously. This is a characteristic of mental disorders.

The results have some limitations. As they are based on cases from psychiatric hospitals in Germany, they are linked to the conditions of the German health system and law. Any decision recommended has to adhere to the specific country's legislation. German legislation may have influenced the selection of cases, so caution is needed when generalising the findings to other health systems. Furthermore, it is not known how the applicants selected the cases that were so challenging that they required an independent ethical review. It cannot be excluded that they may have overlooked other ethically important cases.

Contributors TS has collected the data, did the analysis, developed the model and wrote the manuscript. He is also responsible as a guarantor.

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Competing interests None declared.

Patient consent for publication Not applicable.

Ethics approval According to the Ethics committee of Ulm University, no ethical vote is required for anonymised retrospective analyses of patient files. However, precautions are necessary that individual patients cannot be identified. For this reason, gender, age, place of living, name of hospital and year of presentation are not indicated in the results.

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